

1. Administrative & Study Identification

Full Study Title: A Phase III, Multicenter, Randomized, Double-Masked, Sham-Controlled Study to Investigate the Efficacy, Safety, Pharmacokinetics and Pharmacodynamics of Vamikibart Administered Intravitreally in Patients With Uveitic Macular Edema **Short Title/Acronym:** SANDCAT Study **Protocol Number:** GR44278 **NCT Number:** NCT05642325 **Sponsor Name:** Hoffmann-La Roche **Investigational Product:** Vamikibart (R07200220) **Phase of Development:** Phase III **Medical Monitor:** Hoffmann-La Roche **Clinical Operations Lead**

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the efficacy of vamikibart by assessing the proportion of participants achieving a 15-letter or more improvement from baseline in best-corrected visual acuity (BCVA) at Week 16. **Secondary Objectives:** To evaluate the average change from baseline in BCVA and Central Subfield Thickness (CST) at Week 16, as well as the overall safety, pharmacokinetics, and pharmacodynamics of intravitreal vamikibart.

2.2 Background & Rationale To address the unmet need for a targeted, non-steroid treatment option in patients with Uveitic Macular Edema (UME). UME is the leading cause of moderate to severe vision loss in people with uveitis. The current standard of care relies heavily on corticosteroids, which are associated with significant and serious ocular side effects such as increased intraocular pressure, glaucoma, and cataract formation. Vamikibart targets interleukin-6 (IL-6), a key cytokine in the inflammatory pathway, aiming to resolve macular edema and improve vision while avoiding steroid-associated toxicities.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Sham comparator-controlled **Blinding:** Double-masked **Randomization:** 1:1:1 (High-dose vamikibart vs. Low-dose vamikibart vs. Sham)

3.2 Planned Enrollment & Duration Target \$N\$: 256 participants **Number of Sites:** ~94 locations globally **Estimated Study Start:** 23-Mar-2023 **Estimated Primary Completion:** 07-May-2025

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Diagnosis of macular edema associated with non-infectious uveitis (NIU). 2. Diagnosis of active or inactive, acute, or chronic NIU of any etiology and of any anatomical type (anterior, intermediate, posterior, panuveitis). 3. BCVA letter score of 73 to 19 letters (inclusive) on Early Treatment Diabetic Retinopathy Study (EDTRS)-like charts.

4.2 Exclusion Criteria 1. Evidence of active or latent syphilis infection, tuberculosis infection, or previous/current HIV diagnosis. 2. Uncontrolled intraocular pressure (IOP), glaucoma, or chronic hypotony. 3. Prior use of IVT biologics including anti-VEGFs less than 2-4 months prior to Day 1, or IVT Methotrexate within 4 months prior to Day 1.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Participants receive either high-dose (1 mg) or low-dose (0.25 mg) vamiKibart, or sham treatments. Injections are administered every 4 weeks (Q4W) from Day 1 to Week 12. Following this, dosing is administered as-needed (PRN) from Week 20 to Week 48. **Route of Administration:** Intravitreal (IVT) injection (or sham procedure mimicking an IVT injection). **Duration of Treatment:** Up to 48 weeks of exposure and follow-up.

5.2 Concomitant & Prohibited Therapy Permitted: Standard background medications that do not fall under the exclusionary steroid guidelines. **Prohibited:** Topical corticosteroids and/or topical NSAID \$> 3\$ drops per day in the 14 days prior to Day 1; intraocular or periocular corticosteroid injections in the 2 months prior to Day 1; subconjunctival corticosteroid injection within 1 month prior to Day 1; an OZURDEX implant in the 4 months prior to Day 1; YUTIQ, RETISERT or ILUVIEN implant in the 3 years prior to Day 1.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, Baseline Ophthalmic Assessments, BCVA
Baseline	Day 1	Randomization, First IVT Injection or Sham, Safety Evaluations
Interim	Weeks 4, 8, 12	Q4W IVT Dosing/Sham, BCVA testing, OCT, Safety Monitoring
Primary Endpoint	Week 16	Primary Efficacy Assessment (BCVA improvement), No Dosing
End of Study	Week 48	Final Visual/Anatomical Assessment, Completion of PRN Phase

(Note: During the PRN phase between Week 20 and Week 48, participants are evaluated every 4 weeks to determine if additional treatment is required).

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: The proportion of participants with a 15-letter or more improvement from baseline in best corrected visual acuity (BCVA) at Week 16. **Measurement Method:** Visual acuity testing using EDTRS-like charts.

7.2 Secondary & Exploratory Endpoints * Average change from baseline in BCVA at Week 16. * Average change from baseline in Central Subfield Thickness (CST) at Week 16. * Safety and tolerability profile evaluated through the incidence of treatment-emergent adverse events (e.g., intraocular inflammation, retinal occlusive vasculitis).

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring through comprehensive ophthalmic examinations and patient reports, with a specific focus on procedure-related AEs and intraocular inflammation (IOI). **Serious Adverse Events (SAEs):** Standard protocol requiring investigator reporting to the sponsor within 24 hours of awareness. **Data Monitoring Committee (DMC):** Independent safety monitoring mechanisms implemented as per Hoffmann-La Roche guidelines.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for primary efficacy analysis; Safety Set for all participants receiving at least one dose (or sham) for AE/SAE evaluations. **Sample Size Justification:** A target $N=256$ distributed across three arms (1:1:1), adequately powered ($\alpha = 0.05$) to establish superiority over the sham comparator for the Week 16 BCVA primary endpoint. **Handling of Missing Data:** Last Observation Carried Forward (LOCF) and/or Multiple Imputation (MI) utilized for missing visual acuity and OCT thickness continuous data.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Scheduled onsite and remote clinical monitoring visits to ensure integrity of masked sham procedures and accurate capture of BCVA endpoints. **Audit Trail:** Robust electronic data

capture (eDC), eCRF locking mechanisms, and site query resolution.

11. Study Identifiers & Governance

Primary ID: GR44278 **Registry Number:** NCT05642325 **Sponsor/Lead Organization:** Hoffmann-La Roche **Study Title:** SANDCAT Study **Official Regulatory Title:** A Phase III, Multicenter, Randomized, Double-Masked, Sham-Controlled Study to Investigate the Efficacy, Safety, Pharmacokinetics and Pharmacodynamics of Vamikibart Administered Intravitreally in Patients With Uveitic Macular Edema

12. Clinical Framework (Uveitic Macular Edema Specific)

Primary Condition: Uveitic Macular Edema (UME) **Diagnosis Threshold:** Active or inactive Non-Infectious Uveitis with a baseline BCVA letter score between 73 to 19 (inclusive) **Medical Methodology:** Anti-IL-6 monoclonal antibody therapy **Optimization Target:** Rapid resolution of retinal fluid (CST reduction) and vision recovery (≥ 15 letters gain)

13. Study Procedures & Methodology

Management Pathway: Fixed Q4W IVT induction dosing followed by a stringent criteria-driven PRN maintenance phase. **Education Protocol (HCP):** Site training focused on standardized EDTRS visual acuity measurements and maintaining the double-masked blinding of the sham injection procedure. **Education Protocol (Patient):** Informed consent detailing the risks of IVT injections, potential ocular side effects, and strict adherence to the Q4W visit schedule. **Quality Audit Mechanism:** Central reading center utilized for masking and objective evaluation of CST via Optical Coherence Tomography (OCT).

14. Observation & Follow-Up Schedule

Total Duration: 48 weeks **Onsite Visit Cadence:** Every 4 weeks (Q4W) from Day 1 through Week 48. **Remote Monitoring Frequency:** Routine site and patient follow-ups adhering to standard trial safety governance. **Checklist Review Interval:** Regular review of concomitant ocular therapies (specifically steroids) and BCVA trajectory at every 4-week interval.

15. Sign-off & Approval

Role	Name	Signature	Date		:---	:---	:---	:---
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]					
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]					Lead
Statistician	[Name]	_____	[DD-MMM-YYYY]					